

Pediatric P-10 Pediatric Poisoning / Overdose

Identify substance causing exposure or overdose.
Bring substance container to receiving hospital if safe to do so.
Decontaminate the patient if exposed to hazardous materials.
Consult HazMat responders on handling and decontamination of the patient.
Early notification of receiving hospital

Secure airway using BLS adjuncts if necessary
SpO₂
Oxygen – titrate to SpO₂ >95%
Cardiac Monitor
IV/IO

If patient has
respiratory
distress Refer to
Protocol P-3

Suspected Nerve Agent or
Organophosphate Exposure

Suspected Opioid
Overdose

Suspected Tricyclic
Antidepressant Overdose

Atropine 0.02mg/kg IV/IO
Max single dose 1mg

For moderate to severe exposure
repeat q3 – 5 minutes until response
is achieved

**For patients with respiratory depression AND
unresponsiveness, pinpoint pupils, or other
signs of opioid overdose**

Naloxone 0.1mg/kg IV/IO/IM/IN
Max single dose 1mg
May repeat to max of 2mg

If TCA overdose is suspected and the
following is present:

- QRS complex >100 milliseconds or
ventricular dysrhythmia

Sodium Bicarbonate 8.4% 1mEq/kg IV/IO
for patients ≥2 years old
Contact base for patients <2 years old

Contact Base Hospital for orders on all
other toxic exposures and ingestions for
treatments

DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, Solano County Paramedics may NOT utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.